

Stools - Unusual Color - Female (Adult)

Algorithm 1270 | Adult (18 years and older) | Gender at birth: Female

Definition

- Qatar-localized UAT-only adult and pediatric pathway for black, red, pale or otherwise unusual stool, screening for gastrointestinal bleeding, shock, neonatal/child illness, pregnancy and medication-related mimics; not approved for production.
- Adult patient.
- Female.

Initial assessment

1. [OPEN_TEXT] Describe the color and consistency.
2. [DURATION] How long has this been noticed?
3. [OPEN_TEXT] What medicines, supplements or foods might affect colour, including iron, bismuth, anticoagulants or anti-inflammatory medicines?
4. [OPEN_TEXT] What is the exact age; is the patient pregnant or recently postpartum; and are there dizziness, fainting, pallor, weakness, abdominal pain/swelling, vomiting blood, fever, dehydration, liver disease, bleeding disorder, recent procedure or safeguarding concerns?

Triage and redirect questions

1. Redirect → Diarrhea

Is frequent loose or watery stool the primary concern rather than stool color alone?

Frequent loose or watery stool belongs in the Diarrhea pathway.

1. Disposition level 100

Is bleeding continuing or heavy, are there large clots, vomiting blood, collapse/fainting, confusion, severe weakness, breathing difficulty, marked pallor or severe abdominal pain/swelling?

NHS.UK rectal bleeding guidance lists these as emergency-assessment criteria.

1. Disposition level 70

Is the stool black or dark red, or is there bloody diarrhea?

NHS.UK guidance recommends urgent clinical review for black/dark red stool or bloody diarrhea - possible upper gastrointestinal bleeding.

1. Disposition level 50

Has blood in the stool lasted 3 weeks, has stool consistency changed for 3 weeks, is there pain around the bottom or tummy, tiredness, or unexplained weight loss?

NHS.UK guidance recommends routine primary-care review for these persistent symptoms - occasionally a sign of bowel cancer, so early detection matters.

Advice

Emergency bleeding precautions

- Call Qatar 999 and do not drive. Keep the patient seated or lying down, observed and warm. Give no food, drink or medication while awaiting emergency instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening bleeding; fainting, confusion or other signs of shock.

Urgent stool colour review

- Arrange same-day in-person review through the Qatar pathway approved for UAT for black, dark-red or bloody stool. Do not stop prescribed anticoagulants or other medicines unless a clinician instructs this.
- Call Back If: bleeding increases; dizziness, pallor, weakness or pain develops.

Persistent stool change review

- Arrange in-person primary-care review through the locally approved Qatar pathway. Infants and children require age-specific assessment; medication and food explanations must not be assumed without checking for bleeding or systemic illness.
- Call Back If: new bleeding, pain, fever, vomiting, weakness or dehydration.

First aid

- Call Qatar 999 and do not drive. Keep the patient seated or lying down, observed and warm. Give no food, drink or medication while awaiting emergency instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Rectal bleeding", <https://www.nhs.uk/conditions/rectal-bleeding/> (page last reviewed 12 April 2023)

Provenance

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